جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : ايه خالد حسن على

المحافظة : القاهره **الحي / المدينة :** الاميريه **العنوان :** ب 10 - م 2 مساكن الاميريه

رقم البطاقة : 29309280100986 **رقم التليفون :** 01110401650 **تاريخ تسجيل الإستمارة :** 13-10-2026

العمر(تاريخ الميلاد) : 28-09-1993 **المهنة :** بدون عمل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي
التوقيع:(----------)

المقر بما فيه
التوقيع:(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام

التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: ايه خالد حسن علي
National ID: 29309280100986

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Supportive and palliative Treatment**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين

GUSTAVE ROUSSY
INTERNATIONAL

السيد أ.د / مدير عام المجالس الطبيه

تحيه طيبه وبعد

كشف حساب

بيان بنفقات المريض / ابو خالد حسن

علاج على نفقة الدولة

عن الفتره من 8 / 4 / 2026 الى 4 / 7 / 2026

البيان	رصيد القرار	قيمة المنصرف
الرصيد	3000	
اقامة	0	0
كشف	0	0
اشراف طبي	0	0
اشعه (رنين - مقطعية - اخرى)	0	0
تحاليل طبيه	0	0
مستلزمات	0	0
علاج متنوعه (دم / علاج بنك)	0	0
الادويه	0	0
الإجمالي		20

GRI-E
جوستاف روسي
جوستاف روسي - مصر

الموظف المختص

المراجع



آيه
خالد حسن على
ب ١٠ - م ٢ مساكن الاميرية
الأميرية - القاهره

٢٩٣٠٩٢٨٠١٠٠٩٨٦

KW7275715

Name: ايه خالد حسن على	
MR. : 8945	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 32-year-old individual with a clinical history of recurrent left breast cancer, classified as clinical T3, no metastatic disease identified, invasive ductal carcinoma grade 3, estrogen receptor 8/8, progesterone receptor 8/8, HER2 amplified (2+). Imaging has demonstrated a sternal lesion, a left adnexal cyst, and on MRI of the pelvis performed in April 2026 a small left ovarian hemorrhagic microcyst. A bone scan was negative for metastatic disease. Breast MRI revealed a left retro-areolar and lower-quadrant mass with non-mass enhancement extending to the nipple, associated with nipple discharge and inflammation. The right breast showed a BIRADS 4A lesion; biopsy of this lesion demonstrated fibroadenosis with pseudoangiomatous stromal hyperplasia.

The tumor's biomarker profile is estrogen receptor 8/8, progesterone receptor 8/8, and HER2 amplification. Imaging findings include the left breast mass with nipple involvement, the sternal lesion, and the left ovarian cystic changes, while metastatic workup with bone scintigraphy was negative. The right-breast BIRADS 4A lesion was sampled and found to be fibroadenosis with pseudoangiomatous hyperplasia.

The referring team reported that the patient received neoadjuvant chemotherapy with the TCHP regimen, completing six cycles of TC on 29 August and five cycles of HP on 3 September, and is now being assessed for surgery. The multidisciplinary team recommends proceeding with left-sided skin-sparing mastectomy and immediate silicone reconstruction versus left-sided modified radical mastectomy.

تحريرا في ,

التاريخ :	23-09-2026	**الوقت :**	

الطبيب المعالج :

يعتمد ,

٢٩٣٠٩٢٨٠١٠٠٩٨٦ ٢٠٢٤/١١

بدون عمل

انثى مسلمة ارملة

١ البطاقة سارية حتى ٢٠٣١/١١/٢٩

ا.د. أحمد كمال الدرى
ا.د. محمد أبو الهدى درويش
ا.د. عمر حسين عمر
ا.د. وحيد حسين طنطاوى
ا.د. ياسر عبد العظيم عباس
ا.د. أحمد محمد منيب
أساتذه الأشعة التشخيصية
كلية الطب – جامعة عين شمس

مركز مصر للأشعة
MISR RADIOLOGY CENTER MRC

Patient Name: Aya Khaled Hasan Aly	Patient ID: 1465441
Ref. Phy.: Dear Sir	Study Date: 23/04/2026

MRI OF THE PELVIS (ENDOMETRIOSIS PROTOCOL):-

The study was performed on a high field MR unit.

Technique:-

Multiple pulse sequences were taken before & after IV contrast injection

Findings :

A 32-year-old female patient with known left breast carcinoma started chemotherapy, discovered a left sided endometriotic cyst coming for assessment, the current study reveals:

- The uterus is anteverted, anteflexed, measuring 3.1x3.7x8.3cm in the AP X TS X CC dimensions respectively.
- The endometrial lining is thick yet regular measuring about 9mm.
- The junctional zone is of average thickness measuring about 10mm.
- No evidence of endometrial or myometrial soft tissue lesions.
- The left ovary is bulky, measuring 2.9x3.4x2.6cm in the three orthogonal planes. It shows two small hemorrhagic cysts measuring 1.2cm and 0.8cm in diameter respectively. These hemorrhagic cysts exhibit bright signal on T1 weighted images with low signal on T2 weighted images. No evidence of post contrast enhancement or any suspicious solid components on post contrast images.
- The right ovary measures 1.9x2.3x2.4cm in the three orthogonal planes. It shows multiple small follicles with no solid or cystic lesions.
- No evidence of deep endometriotic plaques within the pouch of Douglas, torus uterinus, uterosacral ligaments, or the uterovesical pouch.
- No sizable pelvic collection.
- No sizable pelvic lymphadenopathies.
- Average filling capacity of the urinary bladder showing no related calculi or masses.
- Normal MR appearance of the rectum and rectosigmoid colon.
- No other remarkable MR abnormalities detected.

OPINION:-

- Small left ovarian hemorrhagic microcysts as detailed above with no suspicious solid components.
- No definite MR evidence of solid endometriotic plaques within the scanned pelvis.

MUCH OBLIGED
ASST. PROF. ALIAA SHEHA,MD,FRCR
PROF.DR.YASSER ABD ELAZIM
K.T.

جوستاف روسي

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

صحة مصر

03/16/2026

MDT FORM

Name:	ايه خالد حسن علي	**MRN:**	8945
Age:	32 Y	**ID:**	29309280100986
Menopausal Sta	pre	**Phone:**	01110401650
NRS SCORE:		**Marital Status:**	widowed
BMI:		**Family History:**	-ve
CO Morbidity:	free	**Reason Of MDT:**	new case

Clinical Data:

BX: IDC G 2

ER: 8 **PR:** 8 **HER2:** 3 **KI67:**

Sonomammography:

CONTRAST MRI
LT BREAST MASS ENHANCEMENT 0.9 * 1.3 CM
REGIONAL PATHOLOGICAL NON MASS ENHANCEMENT OCCUPYING THE WHOLE LOWER QUADRANT EXTENDING TO THE NIPPLE AND ABUTTING THE OVERLYING SKIN WITH THE OVERALL INVOLVEMENT MASS AND NON MASS ENHANCEMENT AREA MEASURING 5 * 13 CM

RT BREAST LIQ NME CORRESPONDING TO MAMMOGRAM FOCAL ASYMMETRY -
----STERIOTACTIC VAB REQUESTED

Mets Workup:

CT CAP
STERNAL ANGLE SMALL SCLEROTIC LESSION
LT ADNEXAL HYPODENSE LESION --US(SIMPLE CYST)

BONE SCAN FREE

History:

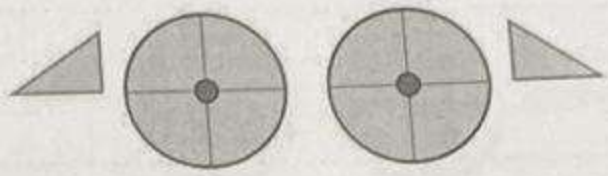
Recommendations:

FOR NACT THEN LT SSM AND SILICON VS LT MRM

MDT Outcome & Plan

FOR NACT TCHP X 6 CYCLES THEN LT SSM AND SILICON VS LT MRM

R L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ نرمين مصطفي	د/ خالد السعدي				

د/ خالد السعدي

[redacted]

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٨٩٤٨

Patient Name	: الأستاذه / ايه خالد حسن	Request No.	: 13005498
Patient No.	: 29309280100986	Request Date	: 17/03/2026
Path No.	: 744/26	Verification Date	: 18/03/2026
Sex	: Female	Financial Category	: جوستاف رومي - اقتصادى
Age	: 32 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical data
Right LIQ breast lesion (non mass enhancement). History of left breast cancer.

Nature of specimen
Tru cut biopsy, collected on 17/3/2026.

Gross examination
Multiple grayish white tissue cores ranging in size from 0.3 cm up to 1.3 cm , totally submitted.

Microscopic examination
Microscopic examination revealed breast tissue showing diffuse stromal fibrosis and focal adenosis. Some of the proliferating ducts are irregularly dilated. The ducts are lined by inner columnar and outer myoepithelial cells, with associated PASH. No atypia or malignancy in sections examined.

Diagnosis
Right breast LIQ lesion , Tru cut biopsy:
Picture compatible with fibroadenosis & Pseudoangiomatous hyperplasia (PASH). No malignancy.

Signature
Dr. Walaa Ibrahim Ghoneim
Dr.Heba Hassan Adly

Verified by :
Dr.Heba Hasan

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

١٩٤٥

PATIENT NAME: AYA KHALED HASSAN ALI **GENDER:** Female **DATE :** 04-Feb-2026 10:17 AM	**PATIENT ID:** 10126277136 **AGE:** 32 y **ORDER:** 3753937

99MTC-MDP BONE SCAN

Desired Information: -
skeletal survey.

Technique:
Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP in the following manner:

Report:

No hot spot that account for osseous deposits could be detected allover the surveyed skeleton.
Degenerative changes are seen over dorso-lumbar vertebrae.
Arthritic changes are seen over both knees.
Both kidneys are normally visualized.

Conclusion:

No definite evidence of metastatic osseous deposits.

Reported By,

Prof. Huda Fathy, Nuclear Medicine Consultant **05-Feb-2026 01:47 PM**

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٤٠ الحي الأول - الشيخ زايد أمام كلية الهندسة
Bone Scan
[[DOCTYPE:BONE_SCAN]]

PATIENT NAME: AYA KHALED HASSAN ALI **GENDER:** Female **DATE :** 17-Feb-2026 03:28 PM	**PATIENT ID:** 10126277136 **AGE:** 32 y **ORDER:** 3770421,3770675

Dynamic Contrast Enhanced MRI Study of the Breast

INDICATIONS:

- Pathological proven left breast invasive duct carcinoma grade III, patient referred for MRI to assess lesion extension.
- Given history of left bloody nipple discharge and recently palpable left lump at left 6 o'clock.

PULSE SEQUENCES:

- Pre-contrast: axial T1,T2,STIR and sagiital T2 WIs
- DWI and ADC mapping
- Post contrast: axial VIBE and Subtraction WIs
- Complementary Ultrasound.

REPORT:

- Prominent fibroglandular parenchyma of both breasts showing moderate back ground parenchymal enhancement.

Left breast:

- **Left breast retroareolar irregular spiculated mass** with altered signal intensity eliciting low T1, intermediate T2 signal and high STIR signal, showing diffusion restriction in DWIS.
- **On post contrast series,** it shows heterogenous mass enhancement, measures 0.9 x 1.3cm ,being 1.5 cm from the nipple, it is the nearest point to the skin is seen closely related to the lateral skin about 3 mm.
- **On plotting time intensity curve** :it shows type 3 washout curve
- Associated Lower quadrant linear NME seen at 6 O'clock along with regional pathological non mass enhancement occupying the whole lower quadrants extending to the nipple and abutting the overlying skin ; with the overall involvement mass and non mass enhancing area measuring 5 x 13 cm.
- **On plotting time intensity curve** :it shows type 2 plateau and type 3 washout curves
- Left UOQ focal delayed non mass enhancement ; likely parenchymal.

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١٠ الحي الأول - الشيخ زايد أمام كلية الهندسة

On plotting time intensity curve : it shows type 1 rising curve.

- Normal skin thickness and contour.
- Normal nipple areola complexes.

Complementary Ultrasound showed:

- Left 6 o'clock retroareolar not circumscribed hypoechoic mass lesion, measures 7x 6 mm, being 1.5cm from the nipple and 3.5 mm from the overlying skin.
- Prominent retroareolar and central ducts. No intraductal soft tissue gross or abnormal vascularity.
- No pathological enlarged axillary or supraclavicular lymph nodes.

Right breast:

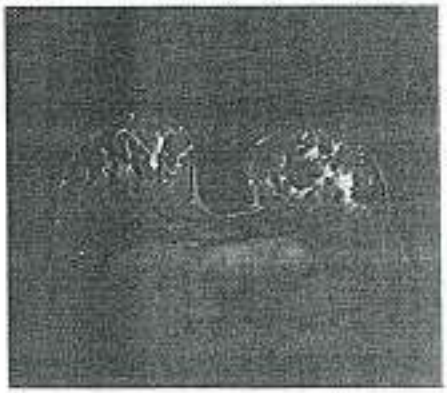
- Right LIQ linear faint non mass enhancement is noted measuring 0.9 x 3cm extending 7cm from the nipple and 1.5cm from the overlying inferior skin. **On plotting time intensity curve** it shows type 1 rising curve (corresponding to focal area of asymmetry at LIQ in the mammogram)
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- **Complementary Ultrasound showed** :
- LOQ glandular parenchyma with no detected suspicious lesions.
- Prominent echogenic glandular parenchyma with prominent anechoic retroareolar ducts with no detected solid or cystic mass lesions
- Non specific axillary lymphadenopathy.

OPINION:

- Left retroareolar pathological proven malignant mass **(BIRADS 6)** associated with lower quadrant regional pathological non mass enhancement extending to the nipple.
- Right LIQ linear NME **(BIRADS 4a)** corresponding to mammogram focal asymmetry for further histopathological correlation by sterotactic VAB in view of contralateral pathology.

Reported By,
Dr. Yasmine Abdel-Naby, MSc
Dr. EMAN FAKER, MD **18-Feb-2026 06:30 PM**

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مستشفى بهية للشفاء والأبحاث
٤ الحي الأول - الشيخ زايد أمام كلية الهندسة



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مستشفى بهية الشيخ زايد
٤٠ الحي الأول الشيخ زايد أمام كلية الهندسة

PATIENT NAME: AYA KHALED HASSAN ALI
GENDER: Female
DATE : 07-Feb-2026 03:29 PM

PATIENT ID: 10126277136
AGE: 32 y
ORDER: 3753935,3753936

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS WITH TRIPHASIC STUDY OF THE LIVER.

TECHNIQUE:

- Multislice helical CT acquisition with automatic IV low osmolar non-ionic contrast medium infusion 60 ml.

FINDINGS:

1-CHEST

- Left lower aspect small enhancing soft tissue nodular lesion is seen measuring 15x10 mm.
- Bilateral subcentimetric axillary lymph nodes.
- No suspicious pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No CT evidence of pathologically enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers.
- No pericardial effusion detected.

2- ABDOMEN AND PELVIS

- The liver is of average size with homogenous CT texture. No focal lesions or intra-hepatic or extra-hepatic biliary radicles dilatation..
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- Left adnexal hypodense lesion measuring 4.4X2.7 cm.
- Right ovarian corpus luteum cyst is seen.

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- Normal appearance of the pancreas, spleen, kidneys (apart from bilateral non-obstructing stones), adrenals, aorta, I.V.C and the other pelvic organs.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No pathologically enlarged abdominal or pelvic lymph nodes.
- No abdominal or pelvic masses or collections.
- **The revised bone window revealed:**

1. No CT evidence of vertebral collapse or extra-osseous soft tissue component.
2. Sternal angle small sclerotic osseous lesion is seen.
3. Bilateral iliac bones sclerotic foci likely bony island.

Opinion:

Left breast pathologically proven IDC grade 3 showing:

- **Small left breast lower aspect soft tissue nodule.**
- **Sternal angle small sclerotic osseous lesion, for bone scan correlation.**
- **No evidence of hepatic deposits.**
- **Left adnexal hypodense lesion, for further US assessment.**

Reported By,
Dr. Yomna Tarek Sayed Moawed
Dr. Mahmoud Ibrahim, MD **09-Feb-2026 02:22 PM**

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١ الحي الأول - الشيخ زايد أمام كلية الهندسة

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name	: الأستاذه / ايه خالد حسن	Request No.	: 13005057
Patient No.	: 29309280100986	Request Date	: 16/02/2026
Path No.	: SR-251/26	Verification Date	: 23/02/2026
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 32 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data
Left breast mass.left nipple discharge.

Nature of specimen
Block and slide revision
Gross examination
One referred paraffin block labelled by (289-25) & six referred slides were received.

Microscopic examination
-Left breast mass:
* Histological type: Invasive duct carcinoma.
* Grade (differentiation +nuclear pleomorphism+ mitosis): (3+3+2) Grade 3
* DCIS: Absent
*Lobular intra epithelial neoplasia : Absent
* Angioinvasion: Absent .
* Microcalcification :Absent.
*Perineural invasion: Absent
• IPT :
* ER: Strong nuclear reaction in 95 % tumor cells (Allred score 8/8)
* PR: Strong nuclear reaction in 95 % tumor cells (Allred score 8/8)
* HER2: Strong complete membrane reaction in >10 % of tumor cells (3+)
* SISH : Not reliable.
-Cytology , left nipple discharge :
-Atypical ductal cells.

Diagnosis
Left breast mass, Block and slide revision
-Invasive duct carcinoma Grade 3 (tubular differentiation 3, pleomorphism 3, mitosis 2)

Verified by :
Dr.Heba Hasan

CAP ACCREDITED
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GUSTAVE ROUSSY
INTERNATIONAL

Patient Name	: الأستاذه / ايه خالد حسن	Request No.	: 13005057
Patient No.	: 29309280100986	Request Date	: 16/02/2026
Path No.	: SR-251/26	Verification Date	: 23/02/2026
Sex	: Female	Financial Category	: جومتالف روسي - نفقة الدولة
Age	: 32 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

(according to Nottingham grading system of breast cancer) , with Duct carcinoma insitu, cribriform, clinging subtypes, intermediate &high grade.
IPT:
* ER: Positive (score 8/8).
* PR: Positive (score 8/8).
* HER2: Positive (score +3).
* SISH : Not reliable.
* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells .IHC 0+:as defined as incomplete membrane reaction in less than 10 % of tumor cells . . IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells

Signature
Dr.Walaa Ibrahim Ghoneim
Dr.Heba El-Shafeey

Verified by :
Dr.Heba Hasan

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

PATIENT NAME: AYA KHALED HASSAN ALI **GENDER:** Female **DATE :** 17-Feb-2026 03:28 PM	**PATIENT ID:** 10126277136 **AGE:** 32 y **ORDER:** 3770421,3770675

Dynamic Contrast Enhanced MRI Study of the Breast

INDICATIONS:

- Pathological proven left breast invasive duct carcinoma grade III, patient referred for MRI to assess lesion extension.
- Given history of left bloody nipple discharge and recently palpable left lump at left 6 o'clock.

PULSE SEQUENCES:

- Pre-contrast: axial T1,T2,STIR and sagiital T2 WIs
- DWI and ADC mapping
- Post contrast: axial VIBE and Subtraction WIs
- Complementary Ultrasound.

REPORT:

- Prominent fibroglandular parenchyma of both breasts showing moderate back ground parenchymal enhancement.

Left breast:

- **Left breast retroareolar irregular spiculated mass** with altered signal intensity eliciting low T1, intermediate T2 signal and high STIR signal, showing diffusion restriction in DWIS.
- **On post contrast series,** it shows heterogenous mass enhancement, measures 0.9 x 1.3cm ,being 1.5 cm from the nipple, it is the nearest point to the skin is seen closely related to the lateral skin about 3 mm.
- **On plotting time intensity curve** :it shows type 3 washout curve
- Associated Lower quadrant linear NME seen at 6 O'clock along with regional pathological non mass enhancement occupying the whole lower quadrants extending to the nipple and abutting the overlying skin ; with the overall involvement mass and non mass enhancing area measuring 5 x 13 cm.
- **On plotting time intensity curve** :it shows type 2 plateau and type 3 washout curves
- Left UOQ focal delayed non mass enhancement ; likely parenchymal.

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On plotting time intensity curve : it shows type 1 rising curve.

- Normal skin thickness and contour.
- Normal nipple areola complexes.

Complementary Ultrasound showed:

- Left 6 o'clock retroareolar not circumscribed hypoechoic mass lesion, measures 7x 6 mm, being 1.5cm from the nipple and 3.5 mm from the overlying skin.
- Prominent retroareolar and central ducts. No intraductal soft tissue gross or abnormal vascularity.
- No pathological enlarged axillary or supraclavicular lymph nodes.

Right breast:

- Right LIQ linear faint non mass enhancement is noted measuring 0.9 x 3cm extending 7cm from the nipple and 1.5cm from the overlying inferior skin. **On plotting time intensity curve** it shows type 1 rising curve (corresponding to focal area of asymmetry at LIQ in the mammogram)
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- **Complementary Ultrasound showed** :
- LOQ glandular parenchyma with no detected suspicious lesions.
- Prominent echogenic glandular parenchyma with prominent anechoic retroareolar ducts with no detected solid or cystic mass lesions
- Non specific axillary lymphadenopathy.

OPINION:

- Left retroareolar pathological proven malignant mass **(BIRADS 6)** associated with lower quadrant regional pathological non mass enhancement extending to the nipple.
- Right LIQ linear NME **(BIRADS 4a)** corresponding to mammogram focal asymmetry for further histopathological correlation by sterotactic VAB in view of contralateral pathology.

Reported By,
Dr. Yasmine Abdel-Naby, MSc
Dr. EMAN FAKER, MD **18-Feb-2026 06:30 PM**

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PATIENT NAME: AYA KHALED HASSAN ALI **GENDER:** Female **DATE :** 07-Feb-2026 03:34 PM	**PATIENT ID:** 10126277136 **AGE:** 32 y **ORDER:** 3753938,3758322

CONTRAST ENHANCED SPECTRAL MAMMOGRAPHY

INDICATION:

Left pathological proven invasive duct carcinoma, grade 3. There is associated DICS component

TECHNIQUE:

- Contrast Enhanced Spectral Mammography CC and MLO views
- IV low osmolar non-ionic contrast medium infusion 100 ml

REPORT:

Inadequate contrast study rendering accuracy of study

- Contrast Enhanced Spectral Mammography showed mild symmetrical back ground parenchymal enhancement.
- Left UOQ moderate non mass enhancement corresponding to glandular tissue
- **Left faint retroareolar linear non mass enhancement ,extending 4 cm posterior to nipple corresponding to dilated retroareolar ducts**
- **No significant mass enhancement is seen corresponding to 6 o; clock index lesion (biopsied lesion)**
- No right breast mass or non mass enhancement.
- Normal skin thickness and contour.
- Normal nipple areola complexes.

OPINION:

Left breast : faint retroareolar linear non mass enhancement with no significant enhancement corresponding to index mass lesion (BIRADS 6) for better assessment with MRI .

- No right breast pathological mass or non mass enhancement (BIRADS 1)

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- **Given history of left nipple bleeding showed Left breast 6:00 indeterminate lesion with single dilated duct as described above. For further histopathological correlation and cytology of the nipple discharge (BIRADS 3)**
- **Right normal breast study (BIRADS 1)**

RECOMMENDATION:
Referral to clinic

Reported By,

Dr. Yasmin Nada, MSc **19-Jan-2026 03:24 PM**

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PATIENT NAME: AYA KHALED HASSAN ALI **GENDER:** Female **DATE :** 19-Jan-2026 02:31 PM	**PATIENT ID:** 10126277136 **AGE:** 32 y **ORDER:** 3733276,3733522

FULL ULTRASOUND EXAMINATION OF BOTH BREASTS with complementary mammography

INDICATIONS:

- Left breast spontaneous uniorificial nipple bleeding.

TECHNIQUE:

- Ultrasound examination of both breasts.
- Complementary bilateral mammography.

REPORT:

Heterogeneous dense breasts with mottling appearance denoting adenosis that may obscure small masses (ACR C).

Left breast :

- No bilateral spiculated mass lesions or grouped microcalcifications.
- Ultrasound showed 6:00 retroareolar dilated duct extending till zone B with area of irregular duct dilatation seen subdermal with internal echogenic contents with peripheral vascularity with color doppler study seen 3 cm from the nipple measuirng 1x0.9 cm seen 0.3 cm from the overlying skin
- Normal skin thickness and contour.
- Few non-specific axillary lymph nodes.

Right breast :

- No bilateral spiculated mass lesions or grouped microcalcifications.
- This was confirmed by Ultrasound which showed No solid or cystic masses.
- Normal nipple areola complexes.
- Normal skin thickness and contour.
- Few non-specific axillary lymph nodes.

OPINION:

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عاجل

DMS

فيسبو

تاكسو تركاربو بلاتينز

تدعيم

[[DOCTYPE:MDT_FORM]]
MDT Form

جوستافروسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

البنك المصري لصحة مصر

10/07/2026

MDT FORM

Name:	ايه خالد حسن علي	**MRN:**	8945
Age:	33 Y	**ID:**	29309280100986
Menopausal Sta		**Phone:***	01110401650
NRS SCORE:		**Marital Status:**	
BMI:		**Family History:**	
Gender			
CO Morbidity:	-	**Reason Of MDT:**	post NACT
Surgical History:*	-	**ECOG Performance Status**	◉ 0 ○ 1 ○ 2 ○ 3 ○ 4

Clinical Data:

BX:

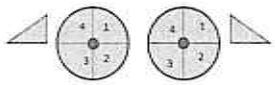
ER: 8/8 **PR:** 8/8 **HER2:** amp **KI67:**

History:

rem left breast cancer ct3 no mo idc 3 er8/8 pr8/8 her22+ sish amplified cts sternal lesion left adnexal cyst .bone scan free mri pelvis 4/2026 small left ovarian haemorrhagic microcyst mri breast left retroaleolar and lower quadrant mass and non mass enhancement extending to nipple (nipple discharge inflamm) rt birads 4a lesion biopsy fibroadenosis pseudoangiomatous hyperplasia

pt received NACT TCHP, completed 6 x TC last on 29/8 and 5 HP on 3/9, for surgical assessment, planned for LT SSM AND SILICON VS LT MRM

R L



T N M

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Sonomammography:

Clinical Staging:

Biomarker/Molecular

Pathological Staging:

Mets Workup:

Recommendations:

FOR LT NSM +SKIN REDUCING +CONTRALATERL SYMETRIZATION SLNX

MDT Outcome & Plan

FOR LT NSM +SKIN REDUCING + SLNB and CONTRALATERL SYMETRIZATION

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. ندى حسن	د. امنية عيسوي		

7/10/2026